



Predicting Patient Stays. Powering Smarter Hospitals.

Series A Fundraise | Confidential

A real-time LOS intelligence layer for operationally smarter hospitals.



The Problem

REACTIVE DISCHARGE PLANNING

Hospitals are flying blind on length of stay.

Without a reliable LOS prediction at admission, capacity plans break down: beds turn over late, elective procedures get delayed, nursing is misallocated, and revenue leaks across the system.

\$30B

annual cost of
unplanned extended
stays

65%

average bed efficiency
across hospitals

CMS

readmission penalties keep
tightening

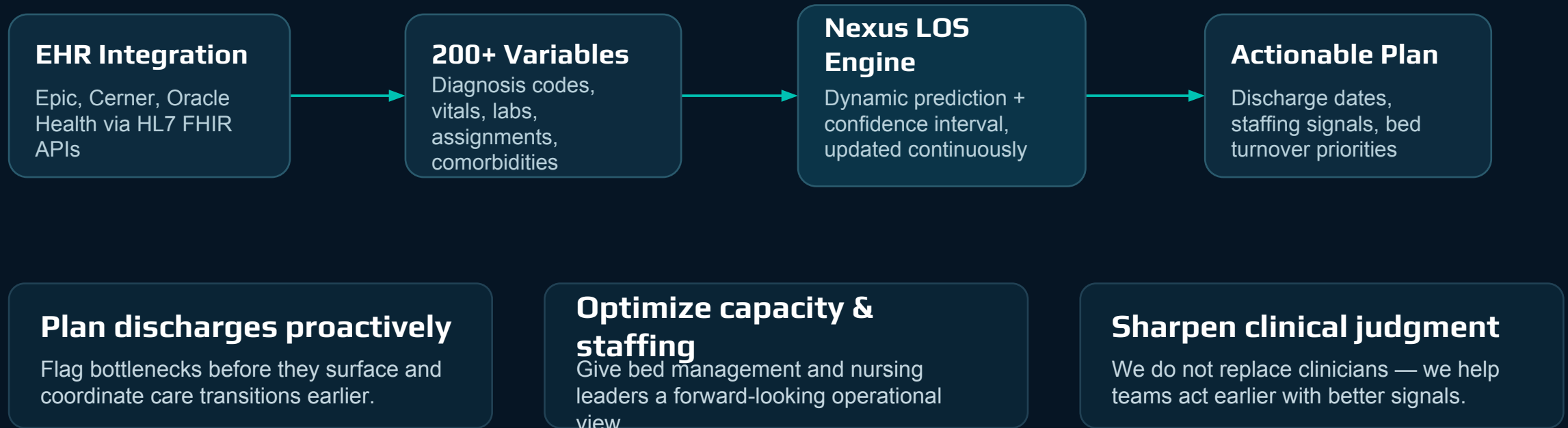
The root cause: clinical teams rely on experience and intuition because there is no data-driven LOS forecast at the point of admission.

No longer good enough.

The Solution

Real-time LOS prediction from admission through discharge.

EHR-NATIVE INTELLIGENCE LAYER



We don't replace clinical judgment. We sharpen it.

Market Size

A large, urgent operational analytics opportunity inside U.S. hospitals.

Operational analytics spend is growing as hospitals digitize capacity management post-pandemic.

TAM / SAM / SOM

TAM \$12.4B

U.S. hospital operational analytics market



SAM \$4.1B

100+ bed hospitals with compatible EHR infrastructure



SOM \$410M

3-year capture target: ~320 hospital clients



19%

market CAGR

3,200

eligible U.S. hospitals

Beachhead customer

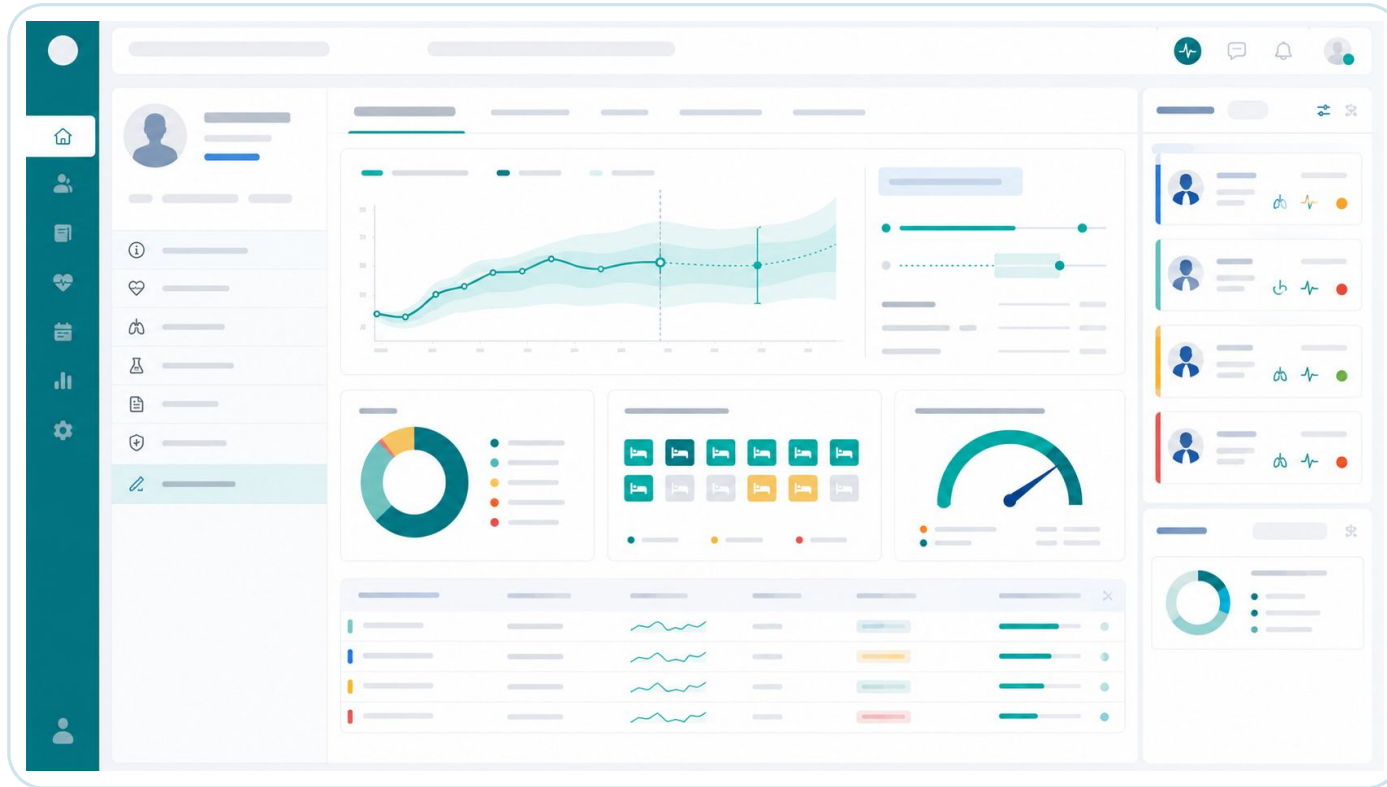
Mid-size regional health systems: complex enough to feel the pain, agile enough to adopt in months rather than multi-year cycles.

3-year target

Capture ~10% of SAM by serving ~320 hospital clients.

Product

Built for the people who actually run hospitals — not just the C-suite.



THREE OPERATIONAL WORKFLOWS

Bed Management Teams

Live dashboard of predicted discharge dates, confidence scores, acuity, and LOS deviation by unit.

Clinical Staff

EHR-embedded alerts flag patients trending toward extended stays 24–48 hours earlier.

Hospital Administrators

Weekly operational intelligence ties predictions to capacity, staffing, and revenue impact.

Epic

Cerner

Oracle Health

HL7 FHIR APIs

Typical deployment: 6–8 weeks

Per-bed, per-year SaaS: \$150K–\$400K annual contracts

Traction

Live, generating revenue, and demonstrating operational ROI.

\$1.2M

ARR across 6 active hospital clients

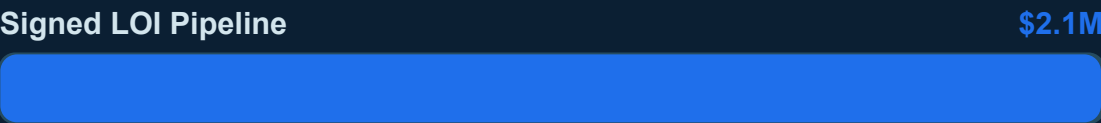
92%

gross retention rate

\$2.1M

signed LOI ARR pipeline from 8 health systems

Revenue momentum



18%

**reduction in unplanned
extended stays**

in two largest deployments within the first 90 days

4-6x

**documented ROI on annual
contract value**

Next acceleration lever: partnership agreement under negotiation with a top-5 national EHR vendor for co-selling.

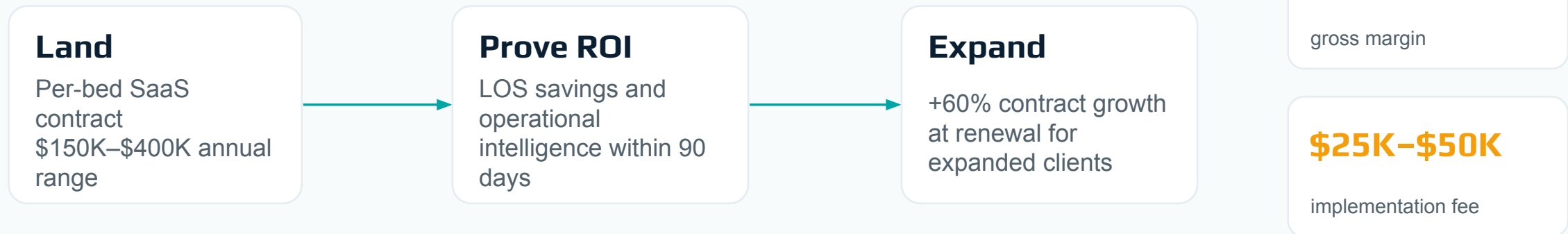
Business Model

Predictable, scalable recurring revenue with expansion built in.

ANNUAL SAAS CONTRACTS BILLED UPFRONT

Revenue is priced per licensed bed, with implementation fees that cover onboarding and integration costs.

Larger health systems are trending toward \$350K–\$400K ACV.



As NexusHealth scales, margin expands: each additional hospital costs a fraction of the initial platform build.

Financials

A \$5M Series A funds growth to breakeven and a stronger Series B position.

Where we are today

\$1.2M

ARR

\$400K

monthly cash burn

~12 mo.

runway at current burn

20%

equity to new investors

The Ask

\$5M Series A at \$20M

pre-money

\$25M post-money valuation

Use of funds

Sales & Marketing

45%

Engineering & Product

30%

Customer Success & Implementation

15%

G&A and working capital

10%

12-month projection: \$4.8M ARR; cash flow breakeven by month 18.

ARR

\$1.2M
Today

\$4.8M
Month 12

\$18M
Year 3

70%+ gross margins by Year 3

Team

Operators, clinicians, and health-tech veterans who have lived the problem.

CEO — Brooke N. Beds

Former VP of Hospital Operations at a 14-hospital regional system; 12 years seeing bed-management dysfunction cost hospitals millions.

CTO — AI Gorithm

ML engineer from a leading health-tech unicorn; led clinical NLP processing 40M+ patient records annually.

Chief Medical Officer — Dr. Paige Rounds

Practicing hospitalist and clinical informaticist; brings credibility and clinical validation to sales.

VP of Sales — Wendy Closedeal

Closed \$30M+ in enterprise SaaS across prior health IT roles; understands how health systems buy.



Advisors

Former hospital CEOs, a healthcare-focused venture partner, and a regulatory consultant with deep CMS reimbursement expertise.

The Ask & Why Now

The window for category leadership in hospital LOS prediction is open.

\$5M

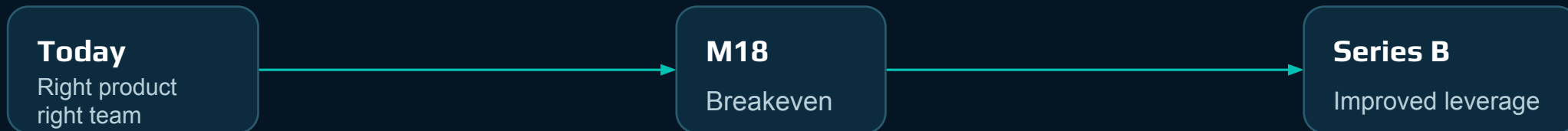
Series A round

at a \$20M pre-money valuation
The capital gets us to cash-flow breakeven, nearly
4x ARR, and a Series B from strength — not
necessity.

Why now

EHR vendors are building adjacent features. Analytics incumbents are starting to notice. Health systems are digitizing operational workflows now.

The company that builds deep integrations and strong retention with health systems in the next 18 months will own this category.



We'd love to build this with you.

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